

Name	: Mr. RAM KEWAL MAHTO	Barcode No	: 241195113
Age/Gender	: 60 Years / Male	Reg. No	: 0012608080172
Referred by	: Dr. SELF	Collected On	: 08-Aug-2026 03:12 PM
Referring Centre	: .	Received On	: 08-Aug-2026 03:22 PM
Sample Tested In	: Whole Blood EDTA	Reported On	: 08-Aug-2026 04:13 PM
Client Address	:	Report Status	: Final Report

HAEMATOLOGY

Test Name	Results	Units	Bio. Ref. Interval	Method
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CBC, COMPLETE BLOOD COUNT

HAEMOGLOBIN	6.4 (C)	gm/dl	13.0-17.0	Photometry (Cyanide free)
HCT (Hematocrit)	22.5	%	40.0-50.0	Numeric Integration
R B C (Red Blood Cell Count)	2.6	millions/cumm	4.50-5.50	Impedance
M C V (Mean Corpuscular Volume)	86.54	fL	83-101	Calculated
M C H (Mean Corpuscular Hb)	24.62	pg	27-32	Calculated
M C H C (Mean Corp Hb Conc)	28.44	g/dL	31.5-34.5	Calculated
RDW (cv)	17.6	%	11.6-14	Calculated
MPV	11.3	fL	7.2-11.7	Calculated
PCT	0.1	%	0.15-0.400	Calculated
PDW-CV	17.3	fL	11.0-20.00	Calculated
PLATELET COUNT	140	x thousand/cumm	150-410	Impedance
TLC (Total Leucocyte Count)	6890	/cumm	4000-10000	Impedance

DIFFERENTIAL LEUCOCYTE COUNT

NEUTROPHIL	80.8	%	40-80	Impedance and absorbance
LYMPHOCYTES	12.3	%	20-40	Impedance and absorbance
EOSINOPHIL	0.5	%	1-6	Impedance and absorbance
MONOCYTES	5.9	%	2-10	Impedance and absorbance
BASOPHILS	0.50		0.0-2.0	Impedance
ABSOLUTE NEUTROPHIL COUNT	5567	/cmm	2000 - 7000	Calculated
ABSOLUTE LYMPHOCYTE COUNT	847	/cmm	1000 - 3000	Calculated
ABSOLUTE EOSINOPHIL COUNT	34	/cmm	20 - 500	Calculated
ABSOLUTE MONOCYTE COUNT	407	/cmm	200 - 1000	Calculated

BLOOD GROUP ABO & Rh

BLOOD GROUP ABO	B
RH FACTOR	POSITIVE

Result rechecked and verified for abnormal cases



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Sr. Consultant

Anjay
Dr. Anjay Sharma
M.D. Pathology
Consultant

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Referring Centre	: .	Received On	: 08-Aug-2026 03:22 PM
Sample Tested In	: Serum	Reported On	: 08-Aug-2026 04:25 PM
Client Address	:	Report Status	: Final Report

BIOCHEMISTRY

Test Name	Results	Units	Bio. Ref. Interval	Method
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KFT WITH ELECTROLYTE SERUM

UREA	92.6 (C)	mg/dL	10.0 - 50.0	Urease
CREATININE	3.60	mg/dl	0.6 - 1.2	Enzymatic PAP
URIC ACID	5.40	mg/dL	3.4 - 7.0	Uricase
BLOOD UREA NITROGEN	43.27	mg/dL	4.6 - 23.36	Urease
SODIUM	135	mmol/L	137 - 145	ISE
POTASSIUM	5.20	mmol/L	3.5 - 5.1	ISE
CHLORIDE	102.00	mmol/L	98 - 107	ISE
eGFR	18.47	ml/min		Calculated

INTERPRETATION:-

1. Causes of increased levels of Urea (or BUN)

High protein diet, Increased protein catabolism,
GI haemorrhage, Dehydration, Congenital heart failure, Renal Failure
Malignancy, Nephrolithiasis, Prostatism

2. Causes of decreased levels of Urea

Liver disease & SIADH

3. Creatinine higher than normal level may be due to:

Blockage in the urinary tract Kidney problems, such as kidney damage or failure, infection, or reduced blood flow
Loss of body fluid (dehydration)

Muscle problems, such as breakdown of muscle fibers

4. Creatinine lower than normal level may be due to:

Myasthenia Gravis & Muscular dystrophy

5. Serum uric acid levels are increased in

Gout, renal failure, inherited metabolic disorders, excess dietary purine intake, Increased nucleic acid turnover (e.g. Leukemia, Myeloma, Radiotherapy, Chemotherapy, Trauma) Psoriasis, preeclampsia and Alcohol Consumption

Biological reference interval : EGFR

Healthy kidney	90 ml/min or above
Mild decrease	60 – 89 ml/min
Moderate decrease	30 – 59 ml/min
Severe decrease	15 – 29 ml/min
Renal failure	Less than 15 ml/min

Note : eGFR calculation based on age, serum creatinine concentration, gender and race. Based on MDRD formula.

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BIOCHEMISTRY

Test Name	Results	Units	Bio. Ref. Interval	Method
LIVER FUNCTION TESTS WITH GGT (LFT+GGT)				
BILIRUBIN TOTAL	0.70	mg/dL	0.0 - 1.0	Diazonium salt (Colorimetric) / Jendrassik
BILIRUBIN DIRECT	0.10	mg/dL	0.0-0.25	Diazonium salt (Colorimetric) / Jendrassik
BILIRUBIN INDIRECT	0.60	mg/dL	0.0-1.1	Calculated
SGOT (AST)	36.40	U/L	0.0-37.0	UV kinetic without PLP (P-5-P)
SGPT (ALT)	40.50	U/L	0.0-40.0 U/L	UV kinetic without PLP (P-5-P)
ALKALINE PHOSPHATASE	218.60	U/L	30--120	PNP AMP kinetic
TOTAL PROTEIN	6.7	g/dL	6.4-8.3 g/dL	Biuret - Colorimetric
ALBUMIN	2.90	g/dL	3.5 - 5.0	Bromocresol Green (BCG)
GLOBULIN	3.8	g/dL	2.3 - 3.5	Calculated
ALBUMIN/GLOBULIN RATIO	0.76		1.0 - 2.3	Calculated
GAMMA GT	70.30	U/L	11.0 - 50.0	Glutamyltransferase



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Sample Tested In	: Serum	Reported On	: 08-Aug-2026 04:35 PM
Client Address	:	Report Status	: Final Report

SEROLOGY

Test Name	Results	Units	Bio. Ref. Interval	Method
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HBsAg SCREEN

HBsAg NON REACTIVE NON-REACTIVE IMMUNOCHROMATOGRAPHY

Comments:

Hepatitis B is a major public health problem of global importance with an estimated 300 million persistent carriers of the hepatitis B virus (HBV) worldwide. Infection with HBV results in a wide spectrum of acute and chronic liver diseases. Epidemiological studies have clearly linked the virus with the development of hepatocellular carcinoma. HBV infection produces an array of unique antigens and antibodies which follow distinct and individual serological patterns. By monitoring these markers it is possible not only to diagnose infection but also to determine the stage of the disease and probable prognosis. HBsAg is the first marker to appear following infection and is the best indirect indicator of potentially infectious sera.

Note : It is screening test only. Reactive samples must be confirmed with other confirmatory tests

HIV I & II SCREENING

HIV I & II ANTIBODY NON REACTIVE NON-REACTIVE IMMUNOCHROMATOGRAPHY

Comments : It is screening test only. Reactive samples must be confirmed with other confirmatory tests like western Blot with fresh sample. Careful interpretation by the referring physician required.

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Sample Tested In	: Serum	Reported On	: 08-Aug-2026 04:52 PM
Client Address	:	Report Status	: Final Report

ELISA

Test Name	Results	Units	Bio. Ref. Interval	Method
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HCV ANTIBODY

HCV ANTIBODY	0.13	COI	0 -- 1.0	ELISA
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(SCREENING TEST ONLY)

Detects human IgG to the HCV proteins i.e. Core, NS3, NS4 and NS5

Limitation of the assay

The current method detects HCV antibody in human serum and plasma is only a screening test. All reactive samples should be confirmed by supplemental assays. Therefore, for a definite diagnosis, the patient's clinical history, symptoms, as well as serological data should be considered. The test detects IgG type of anti HCV. Therefore, samples from acute HCV infection may produce false negative result due to absence of the same as IgG type of anti HCV are produced later during the infection. A non reactive result does not exclude the possibility of exposure to or infection with HCV. False positive results may be encountered in patients with auto immune liver diseases and due to non specific binding of the sample.

Refer conditions of reporting on last page. Tests processed at AM Thyrovision Laboratories PVT. LTD.

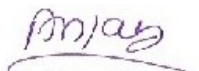
*** End Of Report ***

(Note: The result (C) in this report is critical.)

Conditions of reporting: Test results need correlation with clinical findings and other investigations by a clinician/doctor for diagnosis. Contact lab immediately (within 24 hrs.) if results are unexpected or alarming. The test results pertain to the specimen received from referring lab/clinic/hospital etc. Patient identity to be verified by the sender. Not valid for medico-legal purpose. Any dispute/claims shall subject to the exclusive jurisdiction of Delhi court.



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